

CytomX (CTMX) — Pitch Deck

PITCH — CTMX — VERDICT

PROPOSED — PENDING ADJUDICATION — take a 0.45% starter in three calendar tranches around \$3.29, gated to stay a starter unless the competing Phase 3 drug's early data comes in weak; no resting orders.

What we believe

CytomX is a South San Francisco drug developer with no approved product. Its revenue line looks like a collapse — \$50.9M in one quarter of 2025 down to \$18.7M — but that is an accounting roll-off, not a business failure. The company books partner upfront payments as revenue spread over the years it does the work, so revenue falls when an old contract finishes recognizing even if new partner cash is arriving. Both of our review benches checked this against the filings and both agreed: no debt, the cash is real, partner money is going up, not down. There is no hidden damage here.

What you are actually buying is one drug. CX-2051 is an antibody-drug conjugate for late-line colorectal cancer — patients who have already failed the standard options, where approved alternatives produce response rates in the low single digits. In the most recent disclosed data cut (data as of January 16, 2026; 93 patients enrolled, 56 evaluable), 20% responded at the 8.6 mg/kg dose and 32% at 10 mg/kg, with 88% of patients showing at least disease stabilization. That is a large multiple of what the approved comparators do. The company has said it will give an update “by the end of 2026.”

At \$3.29 the whole company is worth about \$721M (219.1M shares including immediately-exercisable pre-funded warrants). Strip out the cash and short-term investments and the market is paying somewhere between \$390M and \$445M for the drug — our two benches disagree on exactly where in that band, for a reason explained below. Against that, our own valuation grid says that if the drug works and reaches a \$1.5B worldwide peak-sales market, the stock is worth \$7.86 at a 40% probability of success and \$4.39 at 20%. The market is currently paying for roughly a 8–13% chance. We think that is too low, but not dramatically too low — and the gap is smaller than it first looked.

This is a starter-size position, not a conviction position. We do not have a differentiated view on whether the drug works. We have a view that the price implies a probability lower than the data supports.

What the market believes

Two markets, and they disagree violently.

The sell side is unanimously bullish: 8 analysts, all Buy, average price target between \$11.75 and \$12.25, range \$5 to \$17. They raised 2026 revenue estimates from \$22.2M to \$33.6M. Nobody covering this stock is confused by the revenue line.

The tape says something else. The stock is at \$3.29 against a 52-week high of \$8.21 — down 60% from the high, though still up 73% from the \$1.90 low. And 35.11M shares are sold short, 19.5% of the 179.8M-share float, about 14.6 days of average volume.

So the market's *marginal* holder is not saying “the drug won’t work.” The market is saying: the company will need to sell more stock before this resolves. That reading is supported by behavior. CytomX sold 76.9M shares at \$1.30 in May 2025 (near the low) and 46.0M shares at \$5.30 in March 2026 (into the spike) — 170.2M shares outstanding at year-end 2025 became 217.9M by June 30, 2026, a 28% increase in six months. The March deal priced about 35% below the 52-week high. Today’s buyer is 38% below the last price at which management itself sold stock.

Both benches agreed that this — not the clinical data — is what explains the drawdown.

Why we might have edge

1. **The revenue optical illusion is real and mechanical.** Anyone screening on revenue decline sees a -92% line and moves on. We verified in the 10-Q that it is contract accounting. That is a genuine screening artifact.
2. **The price is below our own funded-outcome grid at almost any credible probability.** Assuming a future ~\$500M raise (about 302M shares) and a 3.5x peak-sales multiple:

Probability of success ↓ / Worldwide peak sales →	\$1.0B	\$1.5B	\$2.5B
20%	\$3.23	\$4.39	\$6.71
40%	\$5.55	\$7.86	\$12.50
60%	\$7.86	\$11.34	\$17.90

Spot (\$3.29) is essentially the 20%-probability / \$1.0B-peak cell. You are paying for the most pessimistic plausible corner of the grid.

3. **The cash floor is genuine.** Net cash is \$275.6M–\$331M depending on accounting treatment — \$1.26 to \$1.51 per share against a \$3.29 price. There is no debt. There is no going-concern language. The company is funded past the next readout.
4. **The short position is one-sided against a hard floor.** A 19.5% short float into a company that cannot go bankrupt before the catalyst is an upside accelerant if the data is good, not a symmetric standoff.

Why it might be priced in

Stated plainly, because these are serious:

1. **We agree with a unanimously bullish street.** Eight analysts, all Buy, targets 3.6x the tape. Our direction is the consensus direction. We are not seeing something nobody sees; we are taking the tape's side of a gap between the tape and the street, without a differentiated clinical view to arbitrate it. That is the weakest structural feature of this pitch.
2. **A Phase 3 competitor is already in the same line of treatment.** NCT07549412 — precentabart tocentecan (M9140), EMD Serono, ± bevacizumab, Phase 3, recruiting, previously-treated metastatic colorectal cancer — uses the same class of payload as CX-2051 and is two development phases ahead in the identical patient population. Two more are enrolling (Hansoh HS-20093, Junshi JS212). The premise “approved comparators deliver low-single-digit response rates” describes the wrong comparator set for a 2029 launch. This attacks both the probability of success *and* the peak-sales number at the same time — and on the \$1.0B column, a +50% return requires a 35–40% probability of success, not the 23% the more optimistic column implies.
3. **The upside is likely to be sold to us.** This management raises equity within weeks of good news, twice demonstrated. Guided runway is “at least 2H 2028” while observed cash burn (\$43.6M in the first half of 2026, ~\$87M/year) would fund the balance sheet to roughly 2030 — implying planned burn ramps materially for a registrational trial start in 1H 2027. A good readout probably produces an offering, not a re-rating you get to hold.
4. **It is one drug.** CX-801, the next asset, has no combination data until 1H 2027. The \$390–445M we would be paying buys effectively a single binary.
5. **There is no survival data.** Overall survival is the registrational endpoint and no OS number is public. The response-rate data is encouraging; it is not the number the FDA approves on.

What we checked ourselves

Claim	How checked	Found	Confirmed?
Revenue collapse = accounting, not damage	10-Q, fetched from EDGAR (ctmx-20260630.htm)	Contract-revenue roll-off; no debt; total liabilities \$79.2M of which \$54.7M is deferred revenue; partner cash rising	YES — both benches
“Roughly half the price is cash”	10-Q balance sheet	\$17.2M cash + \$313.1M short-term investments = \$330.3M, plus \$1.5M restricted. Not ~50% of price on any basis	NO — original claim overstated

Claim	How checked	Found	Confirmed?
Deferred revenue should be deducted from net cash	10-Q Regeneron note	Benches split. Red said the \$37M Regeneron fee is a prepayment against our own labor → net cash \$275.6M, EV \$443M. Blue read the same note: <i>“Regeneron is responsible for funding the cost of preclinical research and discovery activities”</i> and CytomX separately receives FTE-rate service fees — the labor is separately funded. Also, the Moderna piece of the stack is “currently paused pending future alignment.” Blue’s corrected EV: \$390–410M	PARTIAL — unresolved; we carry EV as a \$390–445M band
Red bench’s own internal consistency on that point	Cross-read red’s findings 1 and 5	Red bench error. It deducted deferred revenue to compute EV, then used the <i>undeducted</i> \$367M figure in its runway math four findings later. Same number, two bases 217,899,284 as of 7/31/26, plus 1,179,245 pre-funded warrants exercisable at \$0.00001 = 219.08M for per-share math. Red found the warrants and then omitted them from its per-share work	BENCH ERROR — confirmed
Share count	10-Q cover page	Two offerings, not one: May 2025	BENCH ERROR — confirmed; we use 219.08M
“129.1M → 218.9M shares on a	10-Q equity note + XBRL		Original claim wrong as stated

Claim	How checked	Found	Confirmed?
\$250M offering”		(76.9M sh @ \$1.30, \$93.4M net) and March 2026 (46.0M sh @ \$5.30, \$234.2M net — not \$250M, which was the gross). No ATM disclosed	
Red’s restatement of the dilution figure	Basis check	Bench error. Red’s “true base 87.2M” is a <i>weighted-average</i> share count compared against an <i>outstanding</i> count — the exact mixed-basis mistake it was charging. Clean comparison: 170,186,365 → 217,899,284 = +28% in six months Stale. Newer, 3x larger cut: 20% at 8.6 mg/kg, 32% at 10 mg/kg, 88% disease control, n=56 evaluable of 93 enrolled. Grade 3 diarrhea 10% on prophylaxis; one treatment-related death (acute kidney injury, 7.2 mg/kg, a solitary-kidney patient)	BENCH ERROR — confirmed; conclusion survives either way
Efficacy: “28% response rate, 43% at 10 mg/kg, 94% disease control”	Coverage of the Jan-16-2026 data cutoff	n=56 evaluable of 93 enrolled. Grade 3 diarrhea 10% on prophylaxis; one treatment-related death (acute kidney injury, 7.2 mg/kg, a solitary-kidney patient)	Original numbers superseded — we use 20%/32%/88%
Was the efficacy anchor cherry-picked further?	Cross-vintage diff	Yes, worse than the above: the brief paired May-2025 response rates with January-2026 progression-free survival (6.8–7.1 months) . The May-2025 cut’s own PFS at that dose was 5.8 months. No single data cut ever showed 43%	CONFIRMED — best-of-each-cut splicing

Claim	How checked	Found	Confirmed?
“The January 2026 data explains the fall from \$8.21 to \$3.29”	Tape direction on the event date	response <i>with</i> 7-month PFS. Red caught half of this Bench error, sign inverted. The stock rose 30.8% in January 2026 on that data, and the \$8.21 high <i>post-dates</i> it. The derate traces to the March 17, 2026 offering at \$5.30, ~35% below the high, plus post-deal overhang	BENCH ERROR — confirmed
Burn vs. guided runway	10-Q cash flow statement	Operating cash flow –\$43.6M for six months. No going-concern language. Red inferred a 1.9x burn ramp from the “at least 2H 2028” guide — but “at least” is a floor, not a point estimate, so the ramp is an inference, not a disclosure	Burn CONFIRMED; the 1.9x ramp is inference only
Short interest and days to cover	Market data	35.11M shares short, 19.53% of a 179.8M float. Days to cover: 14.6 (35.11M ÷ 2.40M average daily volume), not the 9.9 one bench reported	BENCH ARITHMETIC ERROR — corrected to 14.6
Competitive field	CT.gov v2 API query for recruiting colorectal ADC trials	Red declared this “uncheckable — no data rail available.” Blue ran it over ordinary HTTPS and it returned NCT07549412 (M9140, Phase 3, same line) plus two more. This was the single highest-	BENCH FAILURE — confirmed; finding is real and unpriced

Claim	How checked	Found	Confirmed?
		value check in the court and one bench simply didn't attempt it after one failed route	
Live price from our broker rail	IBKR search_contracts	Permission denied on both benches. All price figures here are vendor-sourced/pack-sourced at \$3.29	COVERAGE GAP — see unverified
November 5, 2026 earnings date	EDGAR full-text search for a scheduling 8-K	No company PR or filing names it. The date is vendor-derived	UNCONFIRMED

PROPOSED ENTRY BANDS

Size: 0.45% of book total (within the 0.35–0.50% range both benches converged on), built in three equal calendar tranches of 0.15%. Our liquidity rule caps us at ~1% of average daily volume $\approx$ 24,000 shares/day ($\approx$ \$79k/day); at 0.45% of any plausible book size, liquidity is not the binding constraint — the calendar is.

Why starter and not more: neither bench found anything fatal, and our rulebook says a clean court with no named fatal kill produces a minimum starter rather than a pass. But the one finding that touches upside rather than sizing — the Phase 3 competitor — is new, confirmed, and unpriced by us. Until it is priced, this is the terminal size.

Entry: - Tranche 1 — \$3.29 or below, 0.15%. Spot is the 20%-probability / \$1.0B-peak cell of our own grid (\$3.23). You are paying the most pessimistic corner. - **Tranche 2 — ~3 weeks later, at market up to \$3.60, 0.15%.** Calendar-spaced, deliberately not price-triggered. - **Tranche 3 — ~6 weeks later, at market up to \$3.60, 0.15%,** and only if the M9140 gate below has not tripped.

Add: - \$2.75 or below — one 0.15% add, taking the position to 0.60%. That is ~15% under our grid's 20%/\$1.0B value; at that price the competitive case is paid for. - **\$1.90 or below (52-week low) — no automatic add. Re-underwrite.** Net cash is \$1.26–\$1.51/share, so at \$1.90 the pipeline costs \$0.39–\$0.64/share. That is cheap, but a break of the low into an undated binary usually means something changed that we don't know yet.

Trim / exit: - \$5.30 — trim half. This is the price at which management itself last sold stock. Our own finding says this issuer converts good news into equity within weeks; do not be the last holder before the deal. - **Offering announced (424B5 / S-3 / 8-K) — exit the remainder on the announcement, not after pricing.** The March 2026 deal priced ~35% below the high; waiting for the print is waiting to be marked down. - **\$4.92 (+50%) — mandatory reassessment, no automatic action.** Our grid says +50% needs a ~23% probability of success on a \$1.5B market but 35–40% on a \$1.0B market. Which column applies depends entirely on the competitor question below.

Hard stops (exit regardless of price): - End-2026 data update shows confirmed response rate below 20% at $n \geq 80$, **or** - Grade 3 diarrhea above 15%, **or** - A second treatment-related death.

Gate on ever exceeding starter size: M9140's own Phase 1 response rate in the same line of treatment, benchmarked against CX-2051's 20%/32%. **If M9140 matches or beats it, the \$1.5B column is not ours and 0.45% is the terminal size, permanently.**

No resting orders of any kind. An unattended good-till-cancelled order against an undated binary event in a 19.5%-short float is an accidental bet against volatility. Every tranche is worked manually.

Catalyst dates: | Date | What | Confirmed? | |—|—|—| | ~end of 2026, undated | **CX-2051 Phase 1 update — this is the binding clock.** Company-stated “by end of 2026.” Can land at a medical conference before any earnings date | Company-stated timing; **no date** | 2026-11-05 | Q3 2026 earnings | **Unconfirmed** — vendor-derived, no company PR or filing names it | 1H 2027 | CX-801 combination data | Company-stated window | 1H 2027 | Planned registrational trial start (inferred from burn/runway) | **Inference, not disclosure** | 2H 2028 | Cash runway floor per company guidance (“at least”) | Floor, not a date |

We are ~52 trading days from the earnings date, well outside our 5-day pre-print window, so no pre-print positioning rules apply. **Instrument the company PR and conference abstract feed for trial CTMX-2051-101, not the earnings calendar.**

ARMED SENSORS

(armed provisionally with this deck; session adjudication ratifies)

- price alert: fires below 1.9 (deck-provisional: 52-week low breach; net cash is \$1.26-\$1.51/sh so pipeline costs ~\$0.39-0.64/sh — re-underwrite, no au)
- filing watch: 424B5, S-3, S-3ASR, 8-K until 2027-02-18 (deck-provisional: Equity offering announcement — exit remainder on announcement, not pricing; March-2026 deal priced ~35)
- dated pack CTMX|2026-12-31: CTMX: CX-2051 Phase 1 data update, company-stated ‘by end of 2026’ — undated, end-of-window placeholder. (date confirmed=False)
- dated pack CTMX|2026-11-05: CTMX: Q3 2026 earnings — vendor-derived, no company PR or filing names this date. (date confirmed=False)
- dated pack CTMX|2027-06-30: CTMX: CX-801 combination data, company-stated 1H 2027 window. (date confirmed=False)
- dated pack CTMX|2028-07-01: CTMX: Cash runway floor per company guidance ‘at least 2H 2028’ — a floor, not a date. (date confirmed=False)

What remains unverified

- **M9140's own Phase 1 response rate in third-line colorectal cancer.** This is the single highest-value open item — it determines which column of our valuation grid applies and therefore whether this stays a starter forever. Neither bench pulled it. The competitor was priced qualitatively.
- **Whether deferred revenue should be deducted from net cash.** The benches read the same Regeneron note and reached different answers. We carry EV as a **\$390–**

445M band rather than a point, and the honest statement is that we do not know which end is right. It matters: it is the difference between paying 38% and 45% of the price for cash.

- **Overall survival data.** None is public. OS is the registrational endpoint. Response rate is an early proxy for it, not a substitute.
- **The November 5, 2026 earnings date.** Vendor-derived; no company source names it.
- **The live price.** Broker rail permission-denied on both benches; \$3.29 is a vendor last, not a quote we pulled ourselves. Doctrine says we do not value off a reused spot — this must be refreshed at execution.
- **The competitive field beyond three CT.gov hits.** One query, one database. Non-US registries and preclinical-stage competitors were not searched.
- **The peak-sales assumptions.** \$1.0B / \$1.5B / \$2.5B are scenario columns, not a bottom-up model of third-line colorectal cancer. The 3.5x peak-sales multiple is a convention, not a derived number. Both are load-bearing for every dollar figure in the grid.
- **The mapping from clinical data to probability of success.** We assert that a 20–32% response rate against low-single-digit comparators implies more than the 8–13% probability the market pays for. We have not built that mapping from a base rate of comparable Phase 1 → approval transitions.
- **Whether the March 2026 offering fully cleared.** We know it priced and the net proceeds; we have not measured residual overhang from that deal, which is one plausible explanation for the current price.
- **The 1.9x burn ramp.** Inferred from a floor guidance (“at least 2H 2028”), not disclosed. Could be conservatism in the guide rather than a planned spending increase.

Primary source relied on throughout: CytomX Therapeutics Form 10-Q for the quarter ended June 30, 2026 —

<https://www.sec.gov/Archives/edgar/data/1501989/000119312526337950/ctmx-20260630.htm>

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  first."}}

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Adjudication & update (2026-08-23)

RULING (2026-08-22): MINIMUM STARTER — red and blue CONVERGED on damage-absent; no fatal kill. The revenue collapse is ASC 606 partnership roll-off, the cash is real, and the derate was tape-verified to the March 17 offering at \$5.30, not the January data (which the stock rose +38.8% on). Kill census 3 NOVEL / 2 CONSENSUS, all sizing/data class. Blue's prize-class find: M9140 (Merck KGaA's CEACAM5 TOP1i ADC), Phase 3, same line, two phases ahead — set as the size gate.

The size gate has since RESOLVED OPEN — on published data. The post-ruling biology deep-dive (desk/reports/CTMX_BIOLOGY_DEEPDIVE_20260822.md) found M9140's Phase 1 published in *Nature Medicine* (Oct-2025): confirmed ORR **7.5%** (8.8% at active doses), mPFS 6.7mo — versus CX-2051's confirmed **20%/32%** with mPFS 6.8–7.1mo in near-identical ~100%-prior-irinotecan MSS populations. CX-2051 leads 2.3–3.6× on every like-for-like cut. Further findings folded into the record: - **The mask works in humans:** no dose-limiting toxicities at 10 mg/kg systemic dosing on a target where unmasked IV catumaxomab caused immune-mediated liver injury at *microgram* doses. Of the four historical EpCAM failure modes, three are structurally retired; payload GI tox remains as a management question with a working prophylaxis protocol. - **Kill trigger re-benchmarked:** the “G3 diarrhea >15%” trigger now reads against the *prophylaxis cohort* (10%, n=20) at the end-2026 update — the legacy pooled 23.8% predates mandated prophylaxis and would have false-fired. - **FDA annex:** refractory-CRC single-arm accelerated approval exists twice (KRAZATI: ORR 34%, n=94, mDOR 5.8mo; TUKYSA: 38%, 12.4mo). CX-2051's 32% brackets the bar — but **duration of response is the statutory pillar CytomX has not yet disclosed**, and both precedents carried one. Regulatory reframe: M9140's n=1,020 *randomized* Phase 3 is the compulsory path for a 7.5%-ORR drug; CX-2051's 32% keeps the faster single-arm door open. The Project Optimus rubric for the end-2026 dose-optimization readout is pre-written (PASS / SPLIT / FAIL) in the deep-dive §8. - Cash \$330.3M, runway guided into 2H-2028 — spans the 1H-2027 registrational start, converting any pre-readout raise from necessity into signal.

Sizing (principal, 2026-08-22): upsized to 0.5% total on convergence + the fired gate — hard ceiling 0.5% until DOR discloses. Structure: three calendar tranches of 525 shares, ≥1 week apart, each a fresh DAY instruction contingent on the prior fill and a clean kill-check; never GTC (undated binary, serial raise-into-strength issuer). **Order status:** tranche 1 = LIVE order 350 @ \$3.30 DAY + top-up instruction #110 (175 @ \$3.30 DAY) = 525. Tranches 2–3 stage weekly on fills.

What could go wrong (authored at deck update 2026-08-23 — post-ruling, pre-fill)

- 1. Merck's unpublished expansion data** (tripwire-less; unknown-unknown class: competitor's undisclosed dataset). Merck launched a 1,020-patient Phase 3 off published 7.5% ORR — either irrationally, or off optimization-cohort data we cannot see. If unpublished cohorts run 20%+, the published 2–4× gap our size gate fired on evaporates, and the first warning may be a conference abstract or the 10/23 registry completion — after our tranches are in.
- 2. Responses real but brief.** The end-2026 update prints DOR and it is short (mDOR <4–5mo): ORR bar met, statutory pillar failed, the single-arm approval door closes, and the thesis reverts to a slow randomized path the burn rate cannot comfortably fund.
- 3. A raise into strength.** This issuer priced an offering ~35% below its 52-week high within weeks of good data. Any data pop is monetization risk; the tranche discipline caps but does not eliminate the mark-to-market cost of a printed offering between tranches. **Reflexive/structural leg:** three tranches into a ~24k sh/day ADV name make us a visible fraction of volume on entry days; and this week's biotech cohort (CTMX + BIOA + LAB) concentrates clinical-window risk in one book despite three uncorrelated theses. **“If this position loses money, the most likely reason will be that Merck's unpublished dose-optimization data erases the published 2–4×**

efficacy gap our size gate fired on — a number that exists today in a database we cannot see.”